Somatic Symptom and Illness Anxiety Disorders (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What it is

In **somatic symptom disorder**, physical symptoms (pain, fatigue, dizziness, stomach problems) are accompanied by excessive thoughts, worry, and behavior about them. In **illness anxiety disorder** (formerly hypochondriasis), the person is preoccupied with having or getting a serious illness even though physical symptoms are absent or mild. Both sit in DSM-5-TR's "Somatic Symptom and Related Disorders" (American Psychiatric Association [APA], 2022).

Key change from older manuals: the symptoms do **not** have to be medically unexplained. The diagnosis rests on the excessive response, not on whether a doctor can find a cause (APA, 2022).

How common: across 32 primary-care studies (70,085 patients), 26% to 35% met criteria for at least one older somatoform diagnosis, and 40% to 49% reported at least one medically unexplained symptom (Haller et al., 2015).

Counselor framing: the symptoms are real, not "all in the head." Treatment aims at living well and worrying less, not at proving there is nothing wrong. Work alongside the person's physician.

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis.

- **Somatic symptom disorder:** one or more distressing or disruptive physical symptoms, plus at least one of: thoughts about their seriousness that are out of proportion; persistently high anxiety about health or symptoms; excessive time and energy devoted to them. Usually lasts more than 6 months. Specifiers: with predominant pain; persistent; mild, moderate, or severe.
- **Illness anxiety disorder:** preoccupation with having or getting a serious illness; physical symptoms absent or mild; high health anxiety and easy alarm; excessive checking and doctor visits **or** avoidance of doctors and hospitals; at least 6 months. Types: care-seeking or care-avoidant.
- **Related diagnoses:** functional neurological symptom (conversion) disorder, psychological factors affecting other medical conditions, and factitious disorder (deliberately falsifying symptoms) belong to the same chapter.

Screening and tracking: the PHQ-15 rates physical symptom burden; scores of 5, 10, and 15 mark low, medium, and high severity, and higher scores track worse functioning and more health care use (Kroenke et al., 2002).

Suicide risk: across 33 studies, 24% to 34% of people with these disorders reported current suicidal thinking and 13% to 67% a past attempt, a risk that appeared independent of other mental disorders (Torres et al., 2021). Ask directly.

When counselors must refer (general clinical consensus): make sure a physician has evaluated the symptoms and stays involved; avoid repeated, unnecessary testing. Refer urgently for suicidality, and for new or changing symptoms that need medical review.

3. Frequently used assessment instruments

These are among the most widely used measures for screening, rating severity, and tracking progress in somatic symptom and illness anxiety disorders. **Screening tools do not diagnose;** a positive screen calls for a clinical interview against DSM-5-TR criteria. Click an instrument's website to find it.

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
PHQ-15 (Patient Health Questionnaire-15)	Physical symptom burden; also an APA DSM-5 Level 2 measure.	Self-report, 15 items	No (free)	None beyond general clinical training	PHQ Screeners site
SSS-8 (Somatic Symptom Scale-8)	Brief measure of physical symptom burden.	Self-report, 8 items	No (free)	None beyond general clinical training	Original article

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
HAI / SHAI (Health Anxiety Inventory, short form)	Severity of health anxiety (illness anxiety).	Self-report, 18 items (short form)	No (free)	None beyond general clinical training	Original article
APA Clinician-Rated Severity of Somatic Symptom Disorder	Rating the excessive thoughts, feelings, and behaviors that define the disorder.	Clinician-rated, brief	No (free from the APA for clinical use)	None beyond general clinical training	APA assessment measures
PHQ-9 (Patient Health Questionnaire-9)	Screening for co-occurring depression.	Self-report, 9 items, about 2 minutes	No (free)	None beyond general clinical training	PHQ Screeners site
GAD-7 (Generalized Anxiety Disorder-7)	Screening for co-occurring anxiety.	Self-report, 7 items, about 2 minutes	No (free)	None beyond general clinical training	PHQ Screeners site
PAI (Personality Assessment Inventory)	Broad clinical and personality assessment for adults, including scales for borderline and antisocial features.	Self-report, 344 items, about 50 to 60 minutes	Yes	★ Level C	PAR

★ **= requires professional qualification to purchase or specialized training.** Publishers sell these tests only to qualified users. At Pearson, **Level B** generally means a master's degree in a related field with formal assessment training, or a health care license; **Level C** means a doctorate in psychology or a related field with formal assessment training, or state licensure in a related field (Pearson qualifications policy). MHS, PAR, and WPS use similar levels. Check each publisher's page before ordering. "Original article" links go to the published study that introduced a free scale that has no official website; the scale can usually be obtained there or from its authors.

4. What works: treatment and the evidence

Bottom line for class: CBT is the best-supported treatment for both disorders, with large effects for illness anxiety and small-to-moderate effects for somatic symptoms. Medication evidence is weak.

Evidence overview (honest effect sizes):

- **Illness (health) anxiety:** across 21 comparisons, CBT had a large effect (d = 1.01) that lasted 6 and 12 months (Cooper et al., 2017). In 19 trials, the effect was moderate to large (g = 0.79), 66% responded, 48% remitted, and internet CBT worked about as well as face-to-face (Axelsson & Hedman-Lagerlöf, 2019).
- **In medical clinics:** among 444 hospital outpatients with health anxiety, adapted CBT doubled the share reaching normal health-anxiety levels at 1 year (13.9% vs. 7.3%) compared with standard care (Tyrer et al., 2014).
- **Somatic symptoms:** across 15 trials (1,671 patients), CBT reduced physical symptoms, anxiety, and depression and improved physical functioning, with lasting benefit, but did not clearly reduce doctor visits (Liu et al., 2019). A Cochrane review of 21 studies found psychological therapies better than usual care, but the effect was small to medium (SMD −0.34) and evidence quality low (van Dessel et al., 2014).
- **Medication (referral notes):** in 26 trials, evidence for newer antidepressants and natural products was of low to very low quality, and older tricyclics did not clearly beat placebo (Kleinstäuber et al., 2014).

CBT for these disorders: how it is used (Cooper et al., 2017; Liu et al., 2019):

- **Build a shared, believable explanation** in which body, attention, worry, and behavior keep symptoms going.
- **Identify and challenge catastrophic beliefs:** "This headache means a brain tumor," "If the doctor didn't find it, they missed it," "I can't function while I'm in pain."
- **Reduce safety behaviors** such as body checking, internet searching, and repeated reassurance-seeking, and gradually face avoided activities.
- **Rebuild activity and meaning** despite symptoms, pacing rather than resting or overdoing.

5. Religion, spirituality, and Christian integration: what the evidence shows

Faith and symptoms: the direction is unclear. In a representative German survey, people with more mental or physical complaints reported **more** spirituality and private religious practice, which suggests that illness may lead people to seek God as much as faith protects against symptoms (Hampel et al., 2019). Among Dutch cancer patients, spirituality did little to lessen the impact of pain and fatigue on distress, though meaning and acceptance may have helped with fatigue (Visser et al., 2018).

How faith is used matters. Across 49 studies, positive religious coping went with better adjustment to stress and negative religious coping with worse (Ano & Vasconcelles, 2005).

Clinicians' own beliefs shape care. Among 1,208 U.S. physicians, more religious doctors were more likely to see unexplained symptoms as a spiritual problem (55% vs. 24%) and to think patients would benefit from attending to their spiritual life (Shin et al., 2013).

Intervention studies. We found **no trial of Christian or religiously integrated CBT for somatic symptom or illness anxiety disorder** (PubMed search, October 31, 2026).

Honest summary for teaching:

1. CBT, delivered in medical settings or online, is the evidence base. Faith integration supports it; it does not replace it.
2. Research does not show that faith protects against these disorders; suffering people may turn to faith more. Positive religious coping is still worth supporting.
3. No faith-integrated trial exists. Avoid telling clients their symptoms are "only spiritual"; that can feel dismissive and delay medical care.

6. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within CBT and alongside medical care. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Listen for catastrophic health beliefs and for spiritual ones: "God is punishing me with this illness," "If I had enough faith I would be healed," "If I stop checking, something terrible will happen" (Ano & Vasconcelles, 2005).
- **Challenge (clinical).** Weigh the evidence for catastrophic predictions, test what happens when checking and reassurance-seeking are reduced, and separate trusting God from demanding certainty about health (Cooper et al., 2017).
- **Replace (pastoral).** Offer Scripture that steadies worry and honors the body:
  - "My times are in Your hands…" (Psalm 31:15). Trusting God with what cannot be known or controlled.
  - "Therefore do not worry about tomorrow, for tomorrow will worry about itself." (Matthew 6:34)
  - "Be anxious for nothing, but in everything, by prayer and petition, with thanksgiving, present your requests to God. And the peace of God… will guard your hearts and your minds in Christ Jesus." (Philippians 4:6–7). Prayer in place of checking.
  - "My flesh and my heart may fail, but God is the strength of my heart and my portion forever." (Psalm 73:26). Meaning and hope even with ongoing symptoms.
- **Faith community (clinical).** Encourage the church to offer practical support without making illness the center of every conversation, and to welcome the person back into service and fellowship as they become more active.
- **Ministers:** take physical complaints seriously, encourage one trusted physician, and avoid either dismissing symptoms or praying in ways that heighten fear about them.

7. Quick classroom takeaways

- Somatic symptom disorder means distressing symptoms plus excessive thoughts, worry, or effort about them; illness anxiety means fear of serious illness with few symptoms. Symptoms need not be medically unexplained. Ask about suicide.
- CBT has large effects for illness anxiety and smaller effects for somatic symptoms; medication evidence is weak. Coordinate with one physician.
- Challenge catastrophic and "God is punishing me" beliefs, reduce checking and reassurance, and replace them with trust that our times are in God's hands.
- Track physical symptom burden with the PHQ-15 or SSS-8 and health anxiety with the Health Anxiety Inventory.

Sources note

All 14 references were checked against Crossref and PubMed (Oct 31, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record. Assessment instrument links and purchase and qualification levels were checked on the publishers' and distributors' websites, or on Crossref for original articles (Nov 1, 2026).

References

American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders (5th ed., text rev.)*. https://doi.org/10.1176/appi.books.9780890425787

Ano, G. G., & Vasconcelles, E. B. (2005). Religious coping and psychological adjustment to stress: A meta-analysis. *Journal of Clinical Psychology*, *61*(4), 461–480. https://doi.org/10.1002/jclp.20049

Axelsson, E., & Hedman-Lagerlöf, E. (2019). Cognitive behavior therapy for health anxiety: Systematic review and meta-analysis of clinical efficacy and health economic outcomes. *Expert Review of Pharmacoeconomics and Outcomes Research*, *19*(6), 663–676. https://doi.org/10.1080/14737167.2019.1703182

Cooper, K., Gregory, J. D., Walker, I., Lambe, S., & Salkovskis, P. M. (2017). Cognitive behaviour therapy for health anxiety: A systematic review and meta-analysis. *Behavioural and Cognitive Psychotherapy*, *45*(2), 110–123. https://doi.org/10.1017/S1352465816000527

Haller, H., Cramer, H., Lauche, R., & Dobos, G. (2015). Somatoform disorders and medically unexplained symptoms in primary care. *Deutsches Ärzteblatt International*, *112*(16), 279–287. https://doi.org/10.3238/arztebl.2015.0279

Hampel, N., Schauenburg, H., Ehrenthal, J. C., Brähler, E., Baie, L., & Heuft, G. (2019). Religiosität: in ihrem Einfluss auf ängstliche und depressive Symptome sowie Körperbeschwerden und Traumata überschätzt? Eine repräsentative Querschnittstudie [Is the influence of religiousness on fearful, depressive and somatic symptoms and psychic traumatization overestimated? A representative cross-sectional study]. *Zeitschrift für Psychosomatische Medizin und Psychotherapie*, *65*(3), 288–303. https://doi.org/10.13109/zptm.2019.65.3.288

Kleinstäuber, M., Witthöft, M., Steffanowski, A., van Marwijk, H., Hiller, W., & Lambert, M. J. (2014). Pharmacological interventions for somatoform disorders in adults. *Cochrane Database of Systematic Reviews*, *2014*(11), CD010628. https://doi.org/10.1002/14651858.CD010628.pub2

Kroenke, K., Spitzer, R. L., & Williams, J. B. W. (2002). The PHQ-15: Validity of a new measure for evaluating the severity of somatic symptoms. *Psychosomatic Medicine*, *64*(2), 258–266. https://doi.org/10.1097/00006842-200203000-00008

Liu, J., Gill, N. S., Teodorczuk, A., Li, Z.-j., & Sun, J. (2019). The efficacy of cognitive behavioural therapy in somatoform disorders and medically unexplained physical symptoms: A meta-analysis of randomized controlled trials. *Journal of Affective Disorders*, *245*, 98–112. https://doi.org/10.1016/j.jad.2018.10.114

Shin, J. H., Yoon, J. D., Rasinski, K. A., Koenig, H. G., Meador, K. G., & Curlin, F. A. (2013). A spiritual problem? Primary care physicians' and psychiatrists' interpretations of medically unexplained symptoms. *Journal of General Internal Medicine*, *28*(3), 392–398. https://doi.org/10.1007/s11606-012-2224-0

Torres, M. E., Löwe, B., Schmitz, S., Pienta, J. N., Van Der Feltz-Cornelis, C., & Fiedorowicz, J. G. (2021). Suicide and suicidality in somatic symptom and related disorders: A systematic review. *Journal of Psychosomatic Research*, *140*, 110290. https://doi.org/10.1016/j.jpsychores.2020.110290

Tyrer, P., Cooper, S., Salkovskis, P., Tyrer, H., Crawford, M., Byford, S., Dupont, S., Finnis, S., Green, J., McLaren, E., Murphy, D., Reid, S., Smith, G., Wang, D., Warwick, H., Petkova, H., & Barrett, B. (2014). Clinical and cost-effectiveness of cognitive behaviour therapy for health anxiety in medical patients: A multicentre randomised controlled trial. *The Lancet*, *383*(9913), 219–225. https://doi.org/10.1016/S0140-6736(13)61905-4

van Dessel, N., den Boeft, M., van der Wouden, J. C., Kleinstäuber, M., Leone, S. S., Terluin, B., Numans, M. E., van der Horst, H. E., & van Marwijk, H. (2014). Non-pharmacological interventions for somatoform disorders and medically unexplained physical symptoms (MUPS) in adults. *Cochrane Database of Systematic Reviews*, *2014*(11), CD011142. https://doi.org/10.1002/14651858.CD011142.pub2

Visser, A., de Jager Meezenbroek, E. C., & Garssen, B. (2018). Does spirituality reduce the impact of somatic symptoms on distress in cancer patients? Cross-sectional and longitudinal findings. *Social Science and Medicine*, *214*, 57–66. https://doi.org/10.1016/j.socscimed.2018.08.012

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